Sl.No	NURSING ACTION	RATIONALE
9	Pad tubing with gauze pads over ear and inspect skin behind ear periodically for irritation/breakdown	Constant pressure may cause skin breakdown.
10	Inspect patient and equipment frequently for flow rate, clinical condition, level of water in humidifier, etc.	Helps identifying any complications that may arise.
11	Ensure that safety precautions are followed	
12	Wash hands	
13	Document time, flow rate and observations made on patient.	
14	Encourage the patient to breath through his/her nose with mouth closed	Provides for optimal delivery of oxygen to patient.
15	Remove and clean the cannula with soap and water, dry and replace every 8 hours. Assess nares at least every 8 hours.	Presence of cannula causes irritation and dryness of the mucous membrane.

Oxygen concentration will vary on many factors like patient's tidal volume and ventilator pattern.

Special Precautions

1. Never deliver more than 2-3 litres of oxygen to patients with chronic lung disease, e.g. COPD (chronic obstructive pulmonary disease)
2. Check frequently that both prongs are in patient's nares.

Oxygen concentration with flow rates	
Flow rate per minute	**Oxygen concentration**
1 litre/mt	24 to 25%
2 litres/mt	27 to 29%
3 litres/mt	30 to 33%
4 litres/mt	33 to 37%
5 litres/mt	36 to 41%
6 litres/mt	39 to 45%

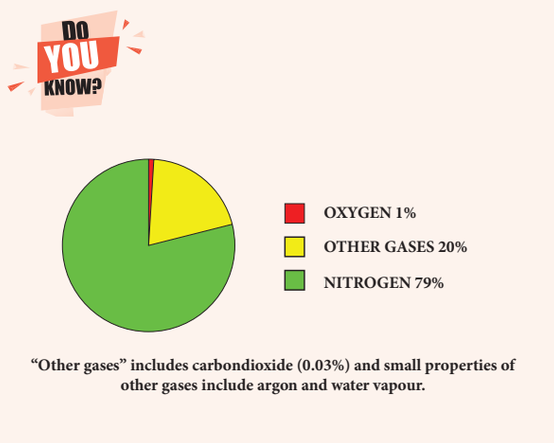
8.2.2 Administering Oxygen By Mask Method

Definition

Administering oxygen to the patient by means of a mask (simple / venturi) according to requirement of patient.

Purpose

1. To relieve dyspnoea.
2. To administer higher concentration of oxygen.



"Other gases" includes carbondioxide (0.03%) and small properties of other gases include argon and water vapour.

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